

EIED

Evidence Integrity & Eligibility Determination

Healthcare AI Benchmark Suite | Version 1.0 | Meredith Reese, RN

Has the AI earned the right to reach this conclusion?

WHAT IT TESTS

Whether an AI can distinguish evidence from assumption and determine if a healthcare decision is supported by sufficient, current, internally consistent, patient-specific, and authoritative evidence.

SUITE AT A GLANCE

- 12 original benchmarks
- 360 fictional source documents
- 48 batch archives
- 12 strict rubrics + 12 design notes

REASONING FRAMEWORK Claim → Evidence → Confidence → Decision

Signature principle: *Labels are claims—not evidence.*

001 Discharge Documentation Closure Eligibility Audit
Clinical documentation integrity and care-transition closure

002 Prior Authorization Evidence Review
Payer criteria, imaging, specialty medication, and DME authorization

003 Clinical Trial Eligibility Validation
Oncology research screening and protocol eligibility

004 Registry Enrollment Audit
Population health registry and colorectal screening outreach

005 Medical Necessity Documentation Review - Post-Acute Services
Home health, DME, wound therapy, oxygen, infusion, and outpatient therapy

006 Medical Necessity Documentation Review - Utilization Management
Imaging, procedures, treatment history, and exception pathways

007 Quality Measure Inclusion Validation
Hypertension quality-measure denominator, exclusion, and numerator review

008 Medication Safety Verification
High-risk medication release, dispensing, and alert verification

009 Case Closure Readiness Audit
Patient safety, communication, privacy, equipment, referral, and follow-up cases

010 Clinical Coding Evidence Validation
Diagnosis and principal-diagnosis coding integrity

011 Longitudinal Evidence Reconciliation
Whole-record reconciliation across multi-setting longitudinal care

012 Clinical Guideline Applicability Audit
Guideline population, eligibility, exclusion, version, and safety gates

DESIGN STRENGTHS

Balanced	Multi-document	Explicit
False-ready and false-hold cases punish blanket approval and blanket escalation.	Every task requires source reconciliation across realistic healthcare artifacts.	Strict rubrics and design notes document evidence, timing, hierarchy, confidence, and failure modes.

Independent portfolio project by Meredith Reese, RN | github.com/reesemerres-jpg